



OFFICIAL

NMHS MHS PROCEDURE

Home and Community Visiting

Scope (Audience)	All NMHS Mental Health Services (MHS) Home and Community Visiting Workers
Scope (Area)	All NMHS MHS Home and Community Visiting Services
Summary	NMHS MHS has a duty of care to provide optimal support and risk management strategies to workers working within community settings, with a view of minimising identified or anticipated risks or threat to the safety of workers. This procedure sets out the mandatory requirements and responsibilities of NMHS MHS whose workers conduct home and community visiting in accordance with the Work Health and Safety Act (2020) , NMHS Work Health and Safety Policy and NMHS Home and Community Visiting Policy
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Communicating for Safety (Std 6)
Policy Sponsors	Community Co Directors & Director of Medical Services
Policy Contact	Adult Community Co Director
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COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

North Metropolitan Health Service (NMHS) Mental Health Services (MHS) are committed to providing a supportive and safe working environment to NMHS MHS workers, who conduct home and community visits.

NMHS MHS has a duty of care to provide optimal support and risk management strategies to workers working within community settings, with a view of minimising identified or anticipated risks or threats to the safety of workers. This document outlines the mandatory requirements and responsibilities of NMHS MHS whose workers conduct home and community visiting.

2. Key Points

NMHS Mental Health Home and Community Services are to ensure that all processes are clearly documented and consistently followed, meeting the following minimum requirements:

- A comprehensive risk assessment should be conducted and document for all consumer visits with appropriate mitigation strategies implemented prior to any work being undertaken.
- Systems must be in place to ensure the real-time location of workers.
- Mechanisms must be available to allow workers to raise an alarm and request assistance when necessary.
- Protocols must be established to initiate appropriate action in any event that a worker cannot be contacted.
- Systems must ensure timely and suitable responses when a worker indicates they require assistance.
- The carrying of duress alarms by workers when community facing is mandatory.

This procedure is to be read in conjunction with the following policies:

- [NMHS Home and Community Visiting Policy](#) which contains Work Health and Safety (WHS) requirements, principles, roles and responsibilities.
- [NMHS Work Health and Safety Policy](#)
- [NMHS Working Alone Policy](#)
- [NMHS Hazard/Incident Reporting and Investigation Policy](#)
- [NMHS MHS Duress Alarm Systems Procedure](#)

3. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Roles and Responsibilities

4.1 Line Managers/Supervisor Responsibilities

Managers and supervisors within NMHS MHS are responsible for:

- Ensuring employees have completed a thorough orientation to the service including local policies and procedures relating to home and community visits **prior** to attending any home or community visit.
- Providing required equipment for worker safety in the community (refer to [8. Fleet Vehicles, Medication/Supplies and other Equipment](#)).
- Allocating a Work Health and Safety (WHS) representative or delegate such as Team Leader who is responsible for auditing and maintaining the equipment.
- Reporting on incidents, accidents and near misses to employees' safety and their clientele in accordance with [NMHS Hazard /Incident Reporting and Investigation Policy](#).
- In collaboration with the clinical team, managers/supervisors have the final decision-making authority regarding whether a high-risk community visit is conducted.

4.2 Worker Responsibilities

It is the role and responsibility of workers/employees of NMHS MHS to promote and protect their safety and the safety of others during community/home visits. In doing so, workers are to:

- Comply with relevant legislation, policies and procedures listed in this document.
- Review and cease/update alerts on PSOLIS regarding risks in line with [MHS Management of Clinical Alerts Policy](#).
- Not attend or withdraw from a home visit if unsafe to proceed with the visit.
- For teams that provide crisis response, access supports as required to attend visits including Western Australian Police Force (WAPOL) attendance.
- Provide an agreed community visiting schedule to their workplace base.
- Report any community home visiting hazards, incidents or accidents.
- **ALWAYS** carry appropriate communication equipment including a personal duress alarm.
- Maintain current training in emergency procedures and feel confident in implementing these protocols in the event of an emergency.
- Prior to each visit, workers must review known safety and risk considerations through a structured information gathering process.

4.3 Students and volunteers

- Students and volunteers will complete the orientation of a new worker relating to community home visiting.
- Attend visits with experienced/senior worker.
- Comply with relevant legislation, policies and procedures listed in this document.
- Report all community home visiting hazards, incidents or accidents.
- **ALWAYS** carry appropriate communication equipment including a personal duress alarm.

5. Worker Safety Assessment and Management of Safety Concerns

Every home and/or community visit should have careful consideration regarding workers' safety. This is due to the dynamics of known and unknown factors associated with working in a community setting that are not limited to the consumer's mental state.

Clinical risk assessment should be undertaken in accordance with the [MHS Clinical Risk Assessment and Management Policy](#).

A worker's safety assessment and planning are an ongoing process of assessing risk and deciding on appropriate visiting plan in collaboration with the treating team and the consumer/carer/family member.

5.1 Worker Safety Assessment

When considering the safety of worker's required to make home and community visits; the decision sits with the individual worker(s), Multidisciplinary Team (MDT), senior(s) and or leadership team members.

When deciding on how to proceed with a home and or community visit, consideration should be given to the following (specifically related to the consumers, their associates and the environment): [Home and Community Visiting Safety Risk Matrix \(Appendix 1\)](#)

The Home and Community Visiting Safety Risk Matrix aims to guide workers, teams and services in how to proceed with home and community visits safely. This should also take into consideration use of relevant policies including the [NMHS Home and Community Visiting Policy](#), and the [MHS Duress Alarm Systems Procedure](#).

In consultation with the MDT and/or their supervisor (after business hours escalated to the relevant on-call nurse coordinator as required), the worker(s) can prioritise their own safety and can withdraw from attending the home and or community visit. This would then require further MDT discussion to ensure clinical care is appropriately managed.

For all initial assessments, and all other visits where a worker is unfamiliar with the consumer, uncertain about their history or has any other misgivings about the consumer, that worker is to arrange to conduct a two-person home visit as detailed within their pre-visit risk review procedures.

6. Procedures

6.1 Prior to planned visit – risk assessment before leaving the workplace

1. All workers undertaking planned home visiting in the community are responsible for undertaking a review of the consumers medical record, Risk Assessment and Management Plan ([RAMP](#)), PSOLIS Alerts to ascertain current risks.
2. For booked appointments with consumers who have a **high** likelihood of a change in presentation/risk the worker is to have direct contact with the consumer to confirm the appointment and screen for any changes in the risk presentation - for example intoxication, animals, COVID, access to/availability of weapons, other persons in attendance.
3. Journey Board /whiteboard /in-out board/staff movement board/shared calendar to be completed including:
 - name of employee.
 - phone number of employees.
 - car registration
 - initials of consumer
 - location of visit

- a home visit risk rating of Low, Medium or High and subsequent related actions/mitigation plan
 - personal Duress Alarm device number
 - expected time of return from the visit
4. Workers are to ensure they have a mobile phone that is fully charged and is turned ON and is unlocked (if this facility is available). Important telephone numbers, such as the Police Service, must be programmed onto the 'speed dial' facility, if it is available, or in contact list. Prior to conducting home visits all workers must familiarise themselves with the correct use of the mobile phones.
 5. Ensure appropriate Person Protective Equipment (PPE) is collected as required and disposal arrangements are in place where applicable.
 6. Prior to a community visit, workers are to review relevant risks outlined below including review of clinical and non-clinical factors using RAMP, PSOLIS alerts, WebPSOLIS, DMR, progress notes and any other relevant documentation.
 7. If risks are identified, then the clinician should have a discussion with their MDT regarding how to proceed using the [Home and Community Visiting Safety Risk Matrix \(Appendix 1\)](#) as a guide. A review should be completed for each visit considering all known factors and notation of unknown factors that have a potential to change the risk.

6.2 Safety precautions on arrival at the location

1. Observe the nearest probable source of help e.g. an occupied home, petrol station.
2. Drive past the address to gain an awareness of the immediate locality and identify possible emergency exits.
3. Park the vehicle where it is clearly accessible. Avoid parking in driveways. Face the vehicle in the direction of an exit route.
4. Always keep the keys for the vehicle, mobile phone and any duress alarms on your person so they are easily accessible. Do not place them in a handbag or case, which you may have to leave in an emergency.
5. Wait for the door to be answered; do not respond to a call of "*come in*". Stand to the side of the entrance and a few paces back after knocking on the door. Discreetly try to gauge whether other persons are present in the home before entering. If this is unclear, ask if there is anyone else at home.
6. Assess the occupational safety and health hazards in the environment such as sounds of domestic altercations, untethered/barking dogs, signs of drug/alcohol abuse. Always have an exit plan with your co-worker should the need arise.
7. If you are concerned at any time, abandon the visit as soon as possible, inform your line manager and the clinician team and return to base.

6.3 Inside the location

1. Always be aware of and maintain appropriate personal space and distance between yourself and the consumer.
2. Look for any potential weapons in the area.
3. When sitting, be careful not to be backed up into a corner and, if possible, maintain a clear view of the door. Place yourself in a position where you have an unobstructed exit.
4. Ensure that you are always seated closer to the exit than the consumer. Never allow a door to be locked behind you. Avoid sitting in a low chair that is difficult to arise from quickly.
5. During the visit, if there are any indications that your safety may be compromised or you sense something is wrong, terminate the visit immediately.

6. If you are unable to leave, call for help via duress/phone and deescalate the situation as best as possible until help arrives. Maintain communication and visual observation of the person whilst removing yourself to a safe environment.
7. If you are behind time with your visits, and you expect to go beyond your estimated return time, advise your designated contact person at the work site of your revised time of return.

6.4 On return to the workplace following a community visit

1. Immediately notify your designated contact person that you have returned or update the Journey Board /whiteboard/in-out board/staff movement board.
2. Return the car keys and duress alarm to charging stations.
3. If you consider the community visit was an **“at risk”** situation, it is important to clearly document the problem/potential hazards you experienced e.g., aggressive dog, lack of privacy to see consumer, alcohol abuse, domestic violence etc. Notify your clinical team about these problems/hazards and document in the consumer’s clinical record.
4. Update the risk alert on PSOLIS to facilitate clear communication of risks to all employees who may be involved in future visits.
5. If you are aware of any other health care workers/NGO’s who are going to visit the consumer at that location, inform them of any hazards identified.
6. Where a consumer is identified as being a high risk for home visits, these visits should be restricted to a **‘two-person visit’** only or consider scheduling at the clinic. In some situations, it may be necessary to cease the home visits until worker safety can be assured.
7. If an incident or injury has occurred, notify your Line Manager verbally, complete a [NMHS Employee Hazard/Incident Form](#) and forward it to your Line Manager. Details of the incident should be entered into the consumer’s clinical record.
8. If a clinical incident occurred, this should be reported in accordance with the [NMHS Clinical Incident Management Policy](#).

6.5 Failure by worker to report back on a community home visit

If **more than 30 minutes** has passed since the expected time of return/contact time and the worker has not previously phoned to change their expected time of return, the following actions are to be taken:

- The team Duty Clinicians/Key Clinicians/identified ‘*phones*’ staff member is to contact the worker by mobile phone. If this is unsuccessful the designated ‘contact person/team’ is to phone the Service Coordinator/Team Leader, and give the workers last known whereabouts, the time of the last call and all other relevant details.
- If the Service Coordinator/Team Leader is unavailable the ‘contact person’ is to notify the Service Manager or SRN7 or Director or other senior member of the Health Service.

6.6 Action to be taken by Management

If the situation is **unknown** or **high risk**, contact the Western Australia Police on 000 and inform them of your concerns and ask them to check the situation.

If the situation is considered low risk, the Service Coordinators/ Team Leaders are to consider a range of strategies:

- Phoning last known community visiting location.
- If situation has not resolved, the Service Coordinator/ Team Leader is to inform the Police Service 131 444 and the Service Manager/ Executive.

Consideration should be given to whether the situation will extend beyond the workers normal working hours, if so, contact workers next of kin.

6.7 Emergency call for assistance

- Any worker may contact the work site and request support and assistance, but this does not preclude direct contact with emergency services via 000 or activation of your duress.
- In an emergency, the worker receiving the call is to obtain the person's name, location and, if possible, the nature of the emergency.
- Based on information given the worker should provide appropriate support. This might include phoning the Police on 000 or other emergency services, to seek urgent assistance to attend the worker's location.
- Contact the worker's Line Manager or, if not available, the nearest, most available, Senior Manager/ Executive.
- Inform the local Health Service Executive about the incident in accordance with the MH Duress Alarms Procedure (under review)
- Consideration should be given to whether the situation will extend beyond the workers normal working hours, if so contact next of kin.

6.8 Activation of duress alarm and response

Workers must never hesitate to activate a person duress alarm (PDA) if their personal safety, or if the safety of others, is compromised.

PDA's are one component in providing a safer work environment. The use of PDA's does not replace the use of comprehensive risk assessment and risk mitigation processes. Workers should use professional judgement to assess if a situation has become unsafe and decide to leave immediately.

1. Activate the duress alarm any time you feel that your safety is at risk.
2. When you need help, press the SOS button for 3-4 seconds till you hear a voice prompt of activating an SOS alarm. Your device will also vibrate at this time.
3. On activation, the Threat Protect Monitoring Centre will receive an alert from the beacon and will call you on the SofiHub shortly afterwards to assess your situation and depending on your needs, will action your emergency as per the instructions provided to us.
4. Activating the SOS also starts the "**Help me!**" text message sequence to your emergency contacts.
5. If safe to do so, the user should clearly verbally state to the attacker that you have activated the duress alarm and clearly state that WA Police will be attending. Keep repeating this statement, as it will ensure the Alarm Receiving service respond immediately and are aware that urgent response is required.
6. Whenever a duress device is activated, the Threat Protect Monitoring Centre receiving an alert the Operator will:
 - a. Listen to the alert and monitor the audio.
 - b. Determine the best course of action.
 - c. Dispatch emergency services (WA Police and/or ambulance) as required (emergency escalation contacts are informed after dispatch of emergency services in a genuine alert).
 - d. Continue to monitor audio until emergency services arrive.
 - e. Call the emergency escalation contacts as assigned to the device.

- f. Threat Protect Monitoring Centre will relay a summary of incident and action taken to the designated contact, including Global Positioning System (GPS) location if a false Alert is suspected.

Consideration should be given to whether the situation will extend beyond the workers normal working hours, if so their next of kin.

7. Home and/or community visits conducted outside of normal working hours

7.1 Adult Community Mental Health Services

1. Fill in the Staff Movement Form (including a list of consumers to be visited in chronological order) and email to the Afterhours handover email. Upon return to the office, an email is to be sent to advise of return.
2. It is the secondary afterhours teams' responsibility to monitor if the other catchment clinicians have sent an email with an estimated return time.
3. If the afterhours staff members have not communicated with the secondary team, by the documented time, the secondary team will confirm safety by:
 - a. Contacting the afterhours telephone number and clinician(s) mobile number.
4. If unable to contact the workers directly - the secondary after hour's team will call the phone number of the community visit (home) provided by the afterhours staff to speak with staff or ascertain time of departure from the consumer's address.
5. If unable to speak with the afterhours staff member through the consumer's contact number, the secondary after hour's team will contact the Police to report concerns for safety of staff members and requested police attend address of community visit.
6. The secondary after hour's team will contact the On-Call Coordinator of Nursing to inform them that they are unable to contact the community staff members on community visit and have contacted Police for assistance.
7. Checks are to continue until contact has been established with the community staff members, or an outcome is communicated from Police. Consideration should be given to whether the situation will extend beyond the workers normal working hours, if so their next of kin should be notified.

7.2 Hospital in the Home (HiTH) – Older Adult

1. HiTH – Older Adult workers who are on duty text their staff movement to the After-Hours Manager on mobile 0476 874 445 and email their departure and expected return time stating patient and area they are visiting as per the staff movement form.
2. The After-Hours Manager is responsible for monitoring if HiTH clinicians have sent a notification within the agreed return-to-base timeframe.
3. Older Adult HiTH staff returning from an afterhours community visit will contact After Hours Manager on Mobile Number 0476 874 445 advising them of their return.
4. It is the responsibility of the afterhours HiTH staff to contact the afterhours manager if they are aware of running late or deferred visit due to other circumstances i.e. incident with patient due to their mental health or physical health.
5. If the afterhours staff members have not communicated with the After-Hours Manager, by the documented time, the After-Hours Manager will confirm safety by:
 - a. contacting the clinician's mobile number.
6. If unable to contact the workers directly:

- a. The After-Hours Manager will call the phone number of the community visit (home) provided by the afterhours staff to speak with staff or ascertain time of departure from the consumer's address.
 - b. If unable to speak with the afterhours staff member through the consumer's contact number, the After-Hours Manager will contact the Police to report concerns for safety of staff members and requested police attend address of community visit.
 - c. The After-Hours Manager will contact the On-Call Coordinator of Nursing to inform them that they are unable to contact the community staff members on community visit and have contacted Police for assistance.
7. Checks are to continue until contact has been established with the community staff members, or an outcome is communicated from Police.
 8. Consideration should be given to whether the situation will extend beyond the workers normal working hours, if so their next of kin should be contacted.

7.3 Hospital in the Home (HiTH) – Youth

1. Youth-HiTH – Workers are required to complete any staff movement on the electronic journey board before departure. This includes the details of:
 - a. the government vehicle registration being used;
 - b. the staff members mobile contact details;
 - c. the consumers contact details including the location for the visit and the mobile/phone contact numbers;
 - d. the PDAs being used for the visit; and
 - e. the estimated time of return to site.
2. The definition of afterhours for Youth-HiTH will be from 17:00 to 20:30 daily. Youth-HiTH teams work across the 7-day week. The Youth-HiTH Duty mobile/phone contact is: 0427 160 426.
3. In the event of a code yellow, or journey board malfunction, home visits will be recorded on a whiteboard in the handover room.
4. After-hours visitations are planned by the MDT and recorded on the electronic journey board, ensuring the identified Person of Contact (POC) (e.g. duty CNS/Shift Coordinator) is kept informed of any updates to provide after-hours support whilst off-site.
5. Youth-HiTH staff returning from an afterhours community visit will advise the identified Person of Contact (e.g. duty CNS/Shift Coordinator) of their return to site as soon as practicable.
6. It is the responsibility of the afterhours Youth-HiTH staff to contact the identified Person of Contact (duty CNS/Shift Coordinator) if they are delayed or if a visit is deferred due to other circumstances i.e. incident with patient due to their mental health or physical health. Risk assessment continues throughout the visiting process prior, during and post. If the worker feels unsafe, they can decide not to attend or withdraw from the visit.
7. Should the Person of Contact (e.g. duty CNS/Shift Coordinator) NOT receive any communication from the Youth-HiTH workers and they are more than 30 minutes past their estimated time of return, the Person of Contact (e.g. duty CNS/Shift Coordinator) will take the following steps:
 - a. contact the worker's mobile number.
 - b. contact the Youth-HiTH site team to check if they have returned but failed to inform the Person of Contact (e.g. duty CNS/Shift Coordinator).
8. If the Person of Contact (e.g. duty CNS/Shift Coordinator) is unable to contact the worker/s directly:
 - a. The Person of Contact (e.g. duty CNS/Shift Coordinator) will call the phone number of the community visit (home) recorded on the journey board (staff movement form) to

Speak with staff or ascertain time of departure from the consumer's address. b. If unable to speak with the worker/s through the consumer's contact number, the Person of Contact (e.g. duty CNS/Shift Coordinator) will contact:

- i. the WA Police to report concerns for safety of staff members, sharing the information provided to them by the Youth-HiTH worker/s prior to departure.
- ii. the Graylands After Hours Clinical Nurse Manager mobile/phone: 0427 321 653 to inform them of the situation and the steps taken.
- iii. the On-Call Coordinator of Nursing (CoN) (identified on the Mental Health Emergency Management Roster) to inform them of the situation and the steps taken. A discussion should be held around when notification of next of kin should occur. The on-call CoN will escalate the emergency response as required.
- iv. checks are to continue until contact has been established with the worker/s or outcome is communicated from Police.
- v. should the Person of Contact (e.g. duty CNS/Shift Coordinator) be due to end their shift and the location of the worker remains unknown the duty of care should be handed over to the Graylands After Hours Nurse Manager/On-Call Coordinator of Nursing. The WA Police should also be updated with Graylands After Hours Nurse Manager/On-Call Coordinator of Nursing contact details to ensure continuity of communication.

7.4 Hospital in the Home (HiTH) – Adult

1. All HiTH workers are required to complete the staff movement boards in the HiTH Offices. These include details of the vehicle used, their contact mobile numbers, the PDA's taken and their expected return times. The definition of outside normal working hours for HiTH Adult Teams will be from 17.00-21.00 daily. Adult HiTH teams work across the 7-day week and are not subject to lower staffing or oversight on weekends between 08.30-1700.
2. Should HiTH workers need to complete an intervention outside of working hours, they need to contact the CNS of the other Adult HiTH site (e.g. Wanneroo contacting SCGH). Should it be the case that both HiTH sites will have interventions requiring them to attend to community locations after 17.00, then they will speak with the SCGH MHU After-Hours Manager. The identified contact, either CNS or After-Hours Manager, will be referred to as the Person of Contact (POC). Prior to departure, HiTH Workers are expected to provide the following information to their POC:
 - a. The Government vehicle registration being used.
 - b. Mobile contact details – recommendation to provide 2 separate phone numbers for redundancy.
 - c. The details of the PDA being used.
 - d. The patient details including the location for the community intervention and rationale for the visit (UMRN, notable risk factors).
 - e. The expected return time.
3. HiTH workers should inform their POC when they have completed their community intervention, are safe and commencing their journey back to their home base. During the intervention if it should become apparent that they may be delayed in their return, they should update their POC as soon as practicable.
4. Should the POC fail to receive any communication from the HiTH workers, and they are more than 30 minutes past their estimated time of return, the POC should take the following steps:
 - a. POC to call the two numbers provided by the HiTH workers.

- b. POC to contact the HiTH site team to determine if they have returned but failed to inform POC.
- c. POC to contact the other HiTH site to check if they have had any contact.
- d. If unsuccessful, POC to contact the WA Police, sharing the information provided to them by the HiTH Workers prior to their departure.
- e. POC to inform the On-Call Coordinator of Nursing to inform them of the situation and the steps taken. A discussion should be held around when notification of next of kin should occur.
- f. Should the POC be a HiTH CNS due to end their shift and the location of the HiTH workers remain unknown, then a report should be provided to the SCGH MHU After-Hours Manager outlining the steps taken and the times involved. The Police should also be updated with the contact details to ensure that there is continuity of communication channels available.

8. Fleet Vehicles, Medication/Supplies and Other Equipment

Medications, consumables and other equipment supplies are to be kept secure at all times. It is the responsibility of the regular home visiting worker/team/service to maintain equipment and supplies. Equipment and consumables must be reviewed for operating effectiveness on a regular basis.

Equipment must be handled in accordance with Manual Tasks and Ergonomics Procedure within the [Work Health and Safety Policy](#)

8.1 Fleet vehicles

Fleet vehicles will be managed in accordance with the [WA Government Fleet Policy and Guidelines](#) and the [NMHS Motor Vehicle Fleet Policy](#).

8.2 Mobile phones

Use of NMHS-owned mobile devices must comply with the [WA Health Acceptable Use of Information and Communications Technology Policy](#).

Use of personal mobile devices for work purposes supports the [NMHS Bring Your Own Device \(BYOD\) Policy](#) and the [NMHS Flexible Work Arrangement Policy](#).

8.3 Medications

Workers are to comply with legislation, Department of Health Operational Directives, and Group/Service policies on the handling and administration of medications; in particular Schedule 8 and Schedule 4 medications outlined within the following [MH Medication: Prescribing, Administration and Management in the Community](#) Policy and [MH Medication: Prescribing and Administration of Antipsychotic Long-Acting Injections Guideline](#)

8.4 Personal protective equipment (PPE)

Refer to the [NMHS Personal Protective Equipment Policy](#) for use, care and maintenance of suitable Person Protective equipment. This policy is a live document and will be updated based on ongoing pandemic advice.

8.5 First Aid Kit

In accordance with the [NMHS First Aid Policy](#), all workers must have access to first aid equipment in government vehicles. These kits are provided and maintained by the NMHS MHS Clinical Equipment Coordinator.

8.6 Bush Fire Survival Kit

In accordance with the [MHS Fire Safety in the Community Procedure](#) a bush fire survival kit will be provided in all government vehicles. Kits are to be supplied and maintained by the NMHS MHS Clinical Equipment Coordinator together with services.

9. Health Records

Health records, and any other documentation, used whilst visiting patients/clients in the community are to be kept confidential and secure, and remain with the worker at all times as outlined within the NMHS Information Management: [Healthcare Record Management Policy](#) and [MHS Healthcare Record Transfer and Transportation Procedure](#).

10. Medical Emergency

If a consumer has a medical emergency during a community visit, workers are to ensure that emergency care is provided commensurate with their expertise, **phone call to 000** and workers will be required to wait for medical treatment to arrive, a summary of this care to be documented in the consumer's medical record.

Workers must be aware of their obligations in line with local area policy, predominantly the [MH Recognising and Responding to Acute Deterioration \(RRAD\) Physiological and Mental State Procedure](#) and the [MH Medical Emergency/Code Blue Community Mental Health Services Procedure](#).

11. Monitoring and Evaluation

Compliance with this procedure is mandatory. There are legislative obligations relating to this procedure where a statutory penalty may apply for non-compliance.

Each NMHS MHS site or service Director and or Service Manager is to ensure compliance with this procedure. This is monitored through the annual Home Visiting Compliance Audit.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed three (3) years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

12. Definitions

Terms	Definitions
Community visit	When a worker attends any setting that is outside the physical location of the hospital or community clinic and is not a consumer's home or personal residence, to provide care or a mental health service. These visits may be undertaken to establish and maintain a therapeutic relationship; facilitate further therapeutic interventions; observe family interactions; enhance assessment and clinical formulation; strengthen liaison with other agencies or service providers; promote consumer engagement; or assertively follow up consumers that may require

	assessment due to a deterioration in their mental health, including assessments under the <i>MHA.2014</i> .
Home visit	When a worker attends a consumer's home or personal residence to provide care or a mental health service.
Hazard	Anything that has the potential to cause harm, damage or adverse health effects to someone or something This could be an object, a person, an animal or a feature of an environment (such layout, noise, temperature, air quality, glare, a work process/task, hazardous substance or exposure, or psychological event).
Incident	An unplanned event (including dangerous occurrences and system failures) resulting in (or having a potential for) injury, ill health, damage or other loss.
Personal Duress Alarm (PDA)	A device that, when activated, alerts identified people or services that a person is under threat and requires immediate assistance. PDAs are wireless devices that are to be worn while conducting home or community visits.
Risk	The probability of injury or harm occurring and the severity of the consequences of that harm.
Risk management	Is the systematic process of identifying, assessing, and mitigating threats or uncertainties that can affect an individual or organisation
Workers	As per worker under the Health Services Act 2016 , this refers to a worker in the NMHS and a person engaged under a contract for services by the NMHS. For the purpose of this procedure this also includes volunteers, students and person on work placement/work experience/unremunerated clinical agreement (UCA).
Workplace	An area, or vehicle under the Work Health and Safety Act 2020 (WHS Act 2020) , where work is carried out for a business or undertaking and includes any place where a worker goes, or is likely to be, while at work for NMHS. This includes but is not limited to hospital premises, laboratories, workshops, training rooms, clinics, onsite and off-site facilities.

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms

13. Related Documents

Legislation	• Mental Health Act 2014 • Health Services Act 2016 • Work Health and Safety Act 2020 • WHS Act 2020
WA Health mandatory policies	• WA Government Fleet Policy and Guidelines • MP 0181/24 Safety Planning for Mental Health Consumers Policy • MP 0155/21 State-wide Standardised Clinical Documentation for Mental Health Services Policy

NMHS Policy documents	• NMHS Home and Community Visiting Policy • NMHS Work Health and Safety Policy • NMHS Working Alone Policy • NMHS Hazard/Incident Reporting and Investigation Policy • NMHS Motor Vehicle Fleet Policy • NMHS Bring Your Own Device Policy • NMHS Flexible Work Arrangement Policy • NMHS Personal Protective Equipment Policy • NMHS First Aid Policy • NMHS Healthcare Record Management Policy • NMHS Smoke Free Policy
NMHS MHS Policy documents	• MHS Duress Alarm Systems Procedure • MHS Medication: Prescribing, Administration and Management in the Community • MHS Medication: Prescribing and Administration of Antipsychotic Long-Acting Injections Guideline • MHS Clinical Record Documentation Procedure • MHS Clinical Risk Assessment Policy • MHS Management of Clinical Alerts Policy

14. Document Control

Document Version Control			
Date	Version	Author	Notes
27/11/2025	1.0	Community Mental Health Service Co Directors	New Procedure developed to replace rescinded MHPHDS Home and Community Visiting Policy and in alignment with the new NMHS Home and Community Visiting Policy. Procedure supports risk management strategies to workers working within community settings, with a view of minimising identified or anticipated risks or threats to the safety of workers. This procedure sets out the mandatory requirements and responsibilities of NMHS MHS whose workers conduct home and community visiting in accordance with the Work Health and Safety Act (2020), NMHS Work Health and Safety Policy.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Appendix 1: Home and Community Visiting Safety Risk Matrix

Community Home Visiting Safety Risk Category	Consumer factors	Environmental Factors
Category Green If safe to do so: 1 Lone worker	- Consumer is known to the service - No recent history of violence or aggression - Not known to regularly misuse illicit substances or alcohol - Stable mental state - Compliant with prescribed medication - (MHS only) - Cooperative with workers - Will abstain from smoking whilst staff visit	- Associates residing with the consumer, have not previously posed a risk to workers - Animals at the property can be removed to another area - Safe access to the property - Within mobile phone reception - During daylight hours - No firearms and / or accessible weapons on the property
Category Amber If safe to do so: 2 workers	- Consumer is not known to the Service - Deterioration in mental state, or fluctuating presentation - Non-compliant with prescribed medication (MHS only) - Recent use of illicit substances but not known to be currently intoxicated	- Property related issues, multiple occupancy, high-rise blocks, multiple units and access/exit issues etc - Access issues: including use of coded entries/exit routes - Known and unknown people residing in any such accommodation as listed above - Outside of business hours or at night - Outside mobile phone reception - Car parking related issues – including use of public carparking spaces, multi-story car parks etc
Category Red For consideration of: Home visit with Police (MH) and 2 workers	- Recent violence or aggression - Intoxicated with drugs or alcohol - Significant clinical acuity - Non-cooperative with health workers - Any other threat to Health worker - Consumer has recently assaulted workers, other health professionals or similar inc WAPOL	- Associates residing with the consumer who potentially pose a risk of harm to workers - Uncontained animals on the property - No safe access to the property - Accessible weapons/ firearms on the property - Associates have recently assaulted workers or police - Unsafe access to the property (for example, 4wd only) - Firearms on the property - Other weapons at the property - Weather including cyclone, flooding, hailstorms and bushfires